Dignity Notes: meeting brief

For Sam's meeting with the owner of East London Care and Support, first week of November 2026. What the app already does for a care agency, what it doesn't do yet, and what to ask.

> **In one line:** carers speak a short note as things happen; Dignity Notes keeps the recording, writes a clear care note, and builds the handover, incident reports, medicines chart and reports from it, so nothing is written twice.

What it does for an agency today

Daily records

- Voice notes turned into tidy, timestamped care notes; the original recording is kept as evidence.
- Additions and corrections are dated and never overwrite the original.
- End-of-shift handover written from the notes; catch-up card for a carer new to a client or back after time away.
- Care plan in 18 standard sections, with every version kept and who agreed to it.

Incidents and safeguarding

- Incidents flagged from the note, with who to tell.
- Incident report form filled in from the note, every amendment kept, print or PDF.
- **Manager review** for agency clients: is a statutory notification needed (CQC Regulation 18 or 16), duty of candour, safeguarding referral, lessons learned.
- Skin integrity forms with follow-up checks until healed.

Medicines

- Medicines chart (MAR): given, refused, missed, when-required doses with limits, time-sensitive doses.
- Every change checked by a second person; training sign-off and certificates.
- Photo of the label fills in the form; box check against the chart; monthly check.
- Works without signal; sent when back online.

Reports and family

- Falls, fluids, food, bowels, urinary, pain and skin reports for any period, printable.
- Family page: daily updates the carer approves, messages, incidents only if agreed.
- Data stored in the EU; who did what, and when, is recorded.

Not there yet (be upfront)

- Pilot only: tested with made-up details; not yet used with real clients.
- No manager dashboard across all clients, rostering, or call monitoring (visit check-in and check-out).
- Doesn't connect to other care management software yet; records can be printed or saved as PDF.
- Still to do before real clients: independent clinical safety review, data protection impact assessment, data processing agreement, NHS Data Security and Protection Toolkit.

Questions to ask

Records and the CQC

- What do inspectors ask to see from your daily records, incidents and medicines?
- Can we have your incident report, skin integrity (body map) and MAR templates?
- How often are MAR charts audited, and by whom?
- How long do you keep records, and in what form?

People and roles

How they work

- Visits or live-in care? How many clients and carers?
- Do carers use their own phones or company phones? Is signal a problem in clients' homes?
- How do handovers and family updates happen now?
- What software do you use now, and what must it connect to?

- Who reviews incidents and decides on notifications (registered manager, coordinator)?
- Who signs off carers' medicines competency, and how often?
- What does a new carer need to know before a first visit?

Data and cost

- Who would be the data controller (the agency)? Any data protection requirements?
- What would make it worth paying for? Per client, per carer or per month?
- Would they try it with made-up details first?

> **Bring back:** their templates (incident, skin or body map, MAR), their top 3 must-haves, who to follow up with, and whether they'd try the pilot with made-up details. Don't share real client details.